

Clinical Study Summary Report (CSSR)

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1. Administrative & Study Identification

Posting ID: 153061325413239510 **Study Title:** A Study of Neoadjuvant Chemotherapy Plus Nivolumab Versus Neoadjuvant Chemotherapy Plus Placebo, Followed by Surgical Removal and Adjuvant Treatment With Nivolumab or Placebo for Participants With Surgically Removable Early Stage Non-small Cell Lung Cancer
Official Title: A Phase 3, Randomized, Double-blind Study of Neoadjuvant Chemotherapy plus Nivolumab versus Neoadjuvant Chemotherapy plus Placebo, followed by Surgical Resection and Adjuvant Treatment with Nivolumab or Placebo for Participants with Resectable Stage II-IIIB Non-small Cell Lung Cancer **Short Title/Acronym:** CheckMate 77T **Protocol Number:** CA209-77T **NCT Number:** NCT04025879 **Sponsor Name:** Bristol Myers Squibb
Investigational Product: Nivolumab (Opdivo) and Chemotherapy
Phase of Development: PHASE3 **Medical Monitor:** BMS Medical Monitor

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To evaluate whether periadjuvant (neoadjuvant, then adjuvant) immunotherapy with nivolumab will prolong event-free survival (EFS) in participants with early-stage non-small cell lung cancer (NSCLC). **Secondary Objectives:** To evaluate pathological complete response (pCR), major pathological response (MPR), overall survival (OS), and safety.

2.2 Background & Rationale Standard treatment with neoadjuvant immunotherapy plus chemotherapy significantly improves outcomes in patients with resectable non-small cell lung cancer (NSCLC). Perioperative treatment (i.e., neoadjuvant therapy followed by surgery and adjuvant therapy) with nivolumab is hypothesized to further reduce the risk of relapse and improve long-term clinical outcomes compared to chemotherapy alone.

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Placebo-controlled **Blinding:** Double-blind **Randomization:** Randomized (1:1) **Trial Status:** ACTIVE_NOT_RECRUITING (Active but not currently recruiting)

3.2 Planned Enrollment & Duration Target \$N\$: 461 participants
Number of Sites: Approximately 110 locations **Estimated Study Start:** 05-Nov-2019 **Estimated Primary Completion:** 26-Jul-2023

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Participants with suspected or histologically confirmed Stage IIA (> 4 cm) to IIIB (T3N2) non-small cell lung carcinoma (NSCLC) with disease that is considered resectable. 2. Treatment-naïve for NSCLC (no prior systemic anti-cancer treatment). 3. Eastern Cooperative Oncology Group (ECOG) Performance Status of ≤ 1 . 4. Ability to provide surgical or biopsy tumor tissue for biomarkers. 5. No brain metastasis.

4.2 Exclusion Criteria 1. Participants with an active, known or suspected autoimmune disease. 2. Any positive test for hepatitis B virus, hepatitis C virus, or human immunodeficiency virus (HIV). 3. Any previous anti-cancer treatment including cytotoxic, IO treatment, targeted agents, or radiotherapy for NSCLC. 4. Prior treatment with any anti-PD-1, anti-PD-L1, anti-PD-L2, or anti-CTLA-4 antibody, or any other antibody or drug specifically targeting T-cell co-stimulation or checkpoint pathways. 5. Known epidermal growth factor receptor (EGFR) mutations or anaplastic lymphoma kinase (ALK) rearrangements.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: Neoadjuvant nivolumab plus platinum-doublet chemotherapy (including Cisplatin / Trade Name: Platinol, ATC Code: L01XA01) every 3 weeks for 4 cycles, followed by surgery, and then adjuvant nivolumab every 4 weeks. **Route of Administration:** Intravenous (IV) **Duration of Treatment:** Neoadjuvant phase (4 cycles), followed by surgery, and up to 1 year of adjuvant therapy.

5.2 Concomitant & Prohibited Therapy Permitted: Standard pre-medications for chemotherapy. **Prohibited:** Concurrent treatment with other investigational agents, concurrent systemic anti-cancer therapies, or immunosuppressive medications.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
Screening	Day -28 to 0	Informed Consent, Medical History, Tumor biopsy, Imaging, Labs
Neoadjuvant Phase	Q3W for 4 cycles	Nivolumab + Chemotherapy dosing, Safety Labs, Tumor Assessment
Surgery	Post-Neoadjuvant	Surgical Resection, Pathological Response Assessment
Adjuvant Phase	Q4W for 1 Year	Adjuvant Nivolumab dosing, Efficacy Assessment, Safety monitoring
End of Study	Post-Treatment	Survival Follow-up, Disease Recurrence Monitoring

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: Event-Free Survival (EFS) by BICR – The length of time from randomization to any of the

following events: progression of disease or worsening of disease precluding surgery, if surgery is attempted but gross resection is abandoned due to unresectable tumor or worsening of disease, progression or recurrence of disease after surgery, progression or recurrence of disease without surgery, or death due to any cause. Progression/recurrence will be assessed by BICR per RECIST 1.1. Participants who do not undergo surgery for reason other than progression will be considered to have an event at RECIST 1.1 progression or death. **Measurement Method:** Assessed by Blinded Independent Central Review (BICR).

7.2 Secondary & Exploratory Endpoints 1. Overall Survival (OS) 2. Pathologic Complete Response (pCR) Rate 3. Major Pathological Response (MPR) Rate 4. Safety and tolerability

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Continuous monitoring via patient assessments, lab tests, and clinical evaluations; Grade 3 or 4 treatment-related adverse events are actively tracked. **Serious Adverse Events (SAEs):** Collected throughout the study (including SARS-CoV-2 related events as added in amendments) and reported within standard regulatory timelines. **Data Monitoring Committee (DMC):** An independent data-monitoring committee providing continuous oversight.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Intent-to-Treat (ITT) for primary efficacy outcomes. Safety Set for AE/SAE evaluations. **Sample Size Justification:** Target sample of $N=461$ properly powered to detect a statistically significant difference in EFS between the nivolumab and placebo perioperative regimens. **Handling of Missing Data:** Standard statistical survival analysis models utilizing censoring techniques for patients without an event at the last known progression-free date.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki.

Monitoring Plan: Supervised by Bristol Myers Squibb and designated Contract Research Organizations (CROs). **Audit Trail:** eCRF locking and tracking of protocol adherence, independent ethics committee/IRB approvals tracked at each trial site.

11. Study Identifiers & Governance

Primary ID: CA209-77T **Registry Number:** NCT04025879 **Posting ID:** 153061325413239510 **Sponsor/Lead Organization:** Bristol Myers Squibb **Study Title:** CheckMate 77T **Official Regulatory Title:** A Phase 3, Randomized, Double-blind Study of Neoadjuvant

Chemotherapy plus Nivolumab versus Neoadjuvant Chemotherapy plus Placebo, followed by Surgical Resection and Adjuvant Treatment with Nivolumab or Placebo for Participants with Resectable Stage II-IIIB Non-small Cell Lung Cancer

12. Clinical Framework (NSCLC Specific)

Primary Condition: Non-Small Cell Lung Cancer (NSCLC) **Disease Areas:** Carcinoma, Non-Small-Cell Lung **Preferred UMLS Name:** Non-Small Cell Lung Carcinoma **Semantic Type:** Neoplastic Process **Definition:** A malignant tumor arising from epithelial cells. Carcinomas that arise from glandular epithelium are called adenocarcinomas, those that arise from squamous epithelium are called squamous cell carcinomas, and those that arise from transitional epithelium are called transitional cell carcinomas (NCI Thesaurus). **Ontology Classifications:** - **Name:** Carcinoma - **MeSH Code:** D002277 - **HPO Codes:** HP:0030731 - **SNOMED ID:** 1187425009 - **SNOMED Hierarchy:** 1187225007 | Malignant epithelial neoplasm (morphologic abnormality), 118285006 | Epithelial neoplasm (morphologic abnormality), 108369006 | Neoplasm (morphologic abnormality), 1240414004 | Malignant neoplasm (morphologic abnormality) **Diagnosis Threshold:** Resectable Stage IIA to IIIB NSCLC **Medical Methodology:** Perioperative Treatment (Neoadjuvant combination + Adjuvant monotherapy) **Optimization Target:** Neoadjuvant pCR/MPR achievement and long-term event-free survival

13. Study Procedures & Methodology

Management Pathway: Standard surgical resection following neoadjuvant chemo-immunotherapy. **Education Protocol (HCP):** Investigator training on recognizing and managing immune-mediated adverse events and surgical timing. **Education Protocol (Patient):** Education on immunotherapy/chemotherapy side effects and perioperative compliance. **Quality Audit Mechanism:** Blinded Independent Central Review (BICR) for primary imaging and central pathology assessments.

14. Observation & Follow-Up Schedule

Total Duration: Up to 1 year of adjuvant therapy plus long-term survival follow-up (median follow-up evaluated at 25.4 months). **Onsite Visit Cadence:** Every 3 weeks (Q3W) during neoadjuvant phase; Every 4 weeks (Q4W) during adjuvant phase. **Remote Monitoring Frequency:** Follow-up contacts for survival and recurrence monitoring per site protocols. **Checklist Review Interval:** End of each treatment cycle.

15. Sign-off & Approval

Role	Name	Signature	Date						
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]						

Clinical Operations Lead | [Name] | ____ | [DD-MMM-YYYY] | | Lead
Statistician | [Name] | _____ | [DD-MMM-YYYY] |